INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Panitumumab + FOLFIRI

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00448
Version 5

Panitumumab and FOLFIRI on Day 1 every 14 days; a 46-hour fluorouracil pump ends around Day 3. No Day 8 infusion.

Treatment stages

01 **A repeating 14-day cycle**
Day ward → home pump → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly. Also report new chest pain, fever or a line/pump problem immediately.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your treatment works

The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

EGFR-TARGETED ANTIBODY

Panitumumab

Vectibix



Blocks growth messages at the cell surface

Panitumumab attaches to EGFR, a receptor on the outside of cells. This can interrupt signals that help some bowel cancers grow. A tumour RAS test helps identify whether this approach is suitable. EGFR also supports healthy skin, which explains why skin and nail changes need attention.

EGFR receives a growth signal → Panitumumab blocks the receptor → Some growth signals are reduced

What to watch for

Report rash, painful cracks or nail changes early. Eye pain or blurred vision, new breathing symptoms, severe diarrhoea, or blistering skin need prompt assessment. Blood tests check minerals such as magnesium.

TOPOISOMERASE I CHEMOTHERAPY

Irinotecan

Campto



Interrupts deoxyribonucleic acid (the cell's genetic material) (DNA) copying

The body converts irinotecan into SN-38. This traps an enzyme called topoisomerase I while DNA is being copied, causing damage that can stop cancer cells dividing. Normal bowel and blood-forming cells can also be affected.

Irinotecan becomes SN-38 → DNA-repair enzyme is trapped → Dividing cells are damaged

What to watch for

Tell the nurse immediately about cramps, sweating or diarrhoea during the infusion. Diarrhoea starting later needs the separate written treatment and contact plan.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

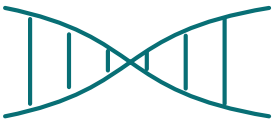
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and review plan

Understand why this option fits your cancer.

The setting matters

Panitumumab + FOLFIRI is used for bowel cancer that has spread and has a confirmed wild-type RAS result. This NCCP regimen covers first-line treatment and selected second-line treatment after a fluoropyrimidine regimen without irinotecan. Your team explains the aim, alternatives and how benefit will be reviewed.

What the tumour tests mean

RAS wild type is required for panitumumab. Your team also considers the cancer location, other molecular results, previous treatment and whether surgery or other local treatment could be useful. A positive eligibility result is not a promise of benefit.

Why this treatment is offered

For selected people whose bowel cancer had already received treatment, adding panitumumab improved tumour shrinkage and delayed progression. A longer overall survival was not established in this analysis.

CHAPTER 02

Your schedule, tests and appointments

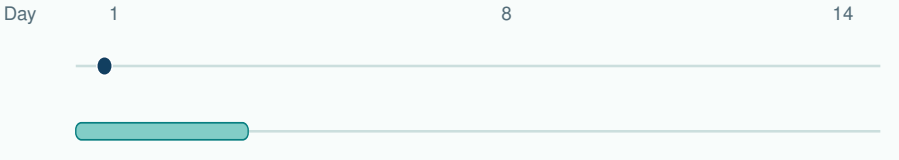
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, review and the next cycle

Panitumumab + FOLFIRI

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY WARD → HOME PUMP → REVIEW

A repeating 14-day cycle

D1 D3 D14

Day 1: panitumumab, irinotecan, folinic acid and fluorouracil, followed by a fluorouracil pump for about 46 hours; disconnection is arranged around Day 3. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

There is no fixed four-cycle course. Scans, symptoms and your priorities guide continuation or changes. With combination treatment, the antibody and chemotherapy may need different holds or changes.

BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. Agree who books each check, reviews results and gives advice if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00448 v5 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Panitumumab + FOLFIRI: treatment and checks

RAS wild-type metastatic colorectal cancer; first-line or selected second-line

A 14-day cycle: panitumumab, irinotecan, folinic acid and fluorouracil start on Day 1, followed by a fluorouracil pump for about 46 hours. Arrange disconnection around Day 3. Continue according to benefit and tolerance.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Blood count, kidney and liver NCCP 00448	Baseline blood count, kidney and liver profile	Review results and symptoms before treatment	Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts	Repeat sooner if unwell or results need follow-up	Follow unresolved abnormalities after treatment
Treatment review NCCP 00448	Confirm exact cancer setting and biomarkers	Explain expected benefit and how it will be assessed	Report new symptoms between visits	Review response, tolerability and active medicines	Agree the next treatment or surveillance plan
Electrolytes and EGFR effects NCCP 00448	Check electrolyte and skin-risk plan	Skin care and prompt reaction reporting	Monitor magnesium, calcium, potassium, skin and eyes	Do not stop follow-up because the infusion stops	Monthly electrolytes, magnesium and calcium for two months after the last panitumumab dose
Heart assessment NCCP 00448	Review cardiac history; baseline electrocardiogram (ECG) if cardiac function is compromised	Report any new chest pain or breathing change	Chest pain, collapse or severe breathing difficulty needs emergency assessment	Additional heart tests only when clinically indicated	Follow any identified heart problem; no fixed routine echo series

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
international normalised ratio (blood-clotting test) (INR) only if taking warfarin NCCP 00448	Tell the team about warfarin; arrange INR monitoring	INR as clinically indicated, rather than a universal weekly schedule	Report bleeding, bruising or significant changes in intake	The prescribing team adjusts warfarin if needed	Continue the individually agreed anticoagulation follow-up
Fluorouracil and pump safety NCCP 00448	DPD testing before fluorouracil unless previously tested; confirm line access	Confirm pump education and disconnection appointment	Review mouth, bowel, cardiac and pump symptoms	Unexpected severe early effects require urgent review	Continue line care until the team removes or changes the device

What each check is for

1 **A repeating 14-day cycle**

Day ward → home pump → review

The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.

2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.

5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Treatment phases and appointments

STEP 2

Checks after treatment

Monthly electrolytes, magnesium and calcium for two months after the last panitumumab dose. Disease assessment follows the consultant plan. Escalate persistent diarrhoea, fever or dehydration between visits. Check INR if taking warfarin, as clinically indicated.

STEP 3

Access and practical support

Central venous access, a 46-hour fluorouracil pump, planned disconnect and emergency contacts. Provide written delayed-diarrhoea instructions separately from atropine for acute cholinergic symptoms.

STEP 4

Paper copies, wallet QR and contacts

The example telephone numbers on the wallet card are placeholders. Your team must replace them with working daytime and out-of-hours numbers before patient use. Keep the card with you; the QR opens the regimen information, not your confidential medical record.

Before you start and before each cycle

The team checks that the plan remains suitable.

Checks and medicines

Review blood counts, kidney/liver function and minerals, current symptoms, allergies and all medicines. DPD testing is needed before fluorouracil unless already documented. Tell the team about severe reactions to previous chemotherapy.

Skin, eyes and breathing

Tell the team about an existing rash, skin infection, dry or painful eyes, contact lenses, or previous lung inflammation. Report new eye pain, light sensitivity or visual change promptly.

When treatment changes or finishes

Follow-up continues after the last infusion.

Monitoring and recovery

Continue your skin plan and planned magnesium, calcium and potassium checks for about eight weeks after panitumumab. New symptoms still need review. Ask when to stop prescribed supportive medicines.

Treatment choices

Your team may pause, reduce, change or stop one or more medicines. Ask which medicine changes, why, and what the next review will assess. Supportive and palliative care can help alongside active treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day-ward medicines	 **Day 1** intravenous (IV)
Fluorouracil pump	 **Days 1–3** 46 hours; disconnect as arranged
Cramps / sweating	 **Day 1** Early
Delayed diarrhoea	 **Days 2–14** Can start after 24 hours; report early
Blood counts / infection	 **Days 1–14** Timing varies; no guaranteed safe days
Mouth / bowel / energy	 **Days 1–14** Record symptoms throughout
Skin / nails	 **Days 1–14** May develop or accumulate over cycles

The planned treatment: Day 1: panitumumab, irinotecan, folinic acid and fluorouracil, followed by a fluorouracil pump for about 46 hours; disconnection is arranged around Day 3. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Blood tests after the final dose: Mineral checks continue for about eight weeks after panitumumab. There is no guaranteed safe day for symptoms.

Skin care continues between visits: Rash or nail changes may build over several cycles. Report symptoms early and follow your written plan.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00448 v5 · official regimen ↗ · Panitumumab product information ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Infusion and pump connection

The team reviews symptoms, blood tests including minerals and your treatment plan. Panitumumab and the day-ward chemotherapy are followed by connection of the fluorouracil pump. Before leaving, confirm the pump-care instructions, disconnection appointment and contact number.

Day 2–3: The pump is delivering treatment

The pump runs for approximately 46 hours from connection. Keep it and the line secure and dry as taught. The team confirms the actual disconnection appointment. Call for a leaking, disconnected or unexpectedly stopped pump. Do not reconnect, alter settings or disconnect it yourself unless specifically trained and instructed.

Day 4–7: After disconnection

Delayed diarrhoea can begin after the first 24 hours and may continue after the pump is removed. Use the written plan and contact the team early. Mouth soreness, nausea and tiredness can overlap. Report poor intake, persistent diarrhoea or mouth pain early. Chest pain or major breathing difficulty requires emergency help.

Day 8–14: Between visits and the next review

Blood counts can fall between visits. Tell the team about bowel symptoms, mouth pain and how you are managing daily activities. Treatment proceeds only after review. Bring the symptom passport and check the next appointment. Day 1 usually repeats 14 days after this cycle started, only if confirmed.

Support around the medicines

1 Panitumumab + FOLFIRI

No routine panitumumab premedication unless previous hypersensitivity/local policy indicates. Written delayed-diarrhoea plan and urgent fever/dehydration advice; do not confuse with acute cholinergic management. Skin care and electrolytes; monthly Mg/Ca/electrolytes for two months after last panitumumab.

2 Act at the first liquid stool

Use the written diarrhoea rescue prescription supplied by your oncology team at the first liquid stool and drink the advised electrolyte-containing fluids. Contact the team early; if no plan was supplied, call immediately. Severe pain, fever, blood, inability to drink or reduced urine needs urgent assessment. Do not treat early infusion cramps or sweating as the same problem.

3 Recovery and the next dose

A delay or reduction is individual. Tell the team about ongoing side effects and previous dose changes; do not assume a calendar date or a stamped card means that treatment can restart.

Living with treatment

Build the plan around what matters to you.

Skin and nails

Ask for your skin-care plan before the first infusion. Use the recommended moisturiser and sun protection; your team may prescribe a cream or an antibiotic. Do not use ordinary acne treatments without advice. Avoid friction on sore skin and tell the team about painful nail folds.

Food, activity and support

Keep fluids and nourishing food within reach. Pace activity around energy levels and ask for dietetic, emotional or practical support. If symptoms prevent eating, drinking or usual tasks, contact the team.

Pregnancy and fertility

Discuss pregnancy plans and fertility before treatment. Panitumumab requires contraception during treatment and for two months afterwards, and breastfeeding should be avoided for that period. When chemotherapy is also included, its precautions may last longer; ask for the instructions for the whole regimen.

Recognise changes early

You do not need to identify the responsible medicine before asking for help.

Track impact, not just a score

Describe when symptoms began, whether they are worsening, and whether they affect food, fluids, sleep, walking or daily tasks. Your team assigns the clinical grade. Use the passport if it helps; it is not required to access this information.

Two bowel-symptom patterns

Cramps, sweating or early diarrhoea during irinotecan need immediate nursing attention. Delayed diarrhoea begins more than 24 hours later and can occur before the next cycle. Follow your written treatment and escalation plan; fever, blood, severe pain or dehydration requires urgent assessment.

Visits, access and coordination

Know what happens in the clinic and what happens at home.

The delivery plan

Day 1: panitumumab, irinotecan, folinic acid and fluorouracil, followed by a fluorouracil pump for about 46 hours; disconnection is arranged around Day 3. A cycle lasts 14 days. Treatment continues while it is helping and side effects remain acceptable.

Radiotherapy or other treatment

This regimen has no routine radiotherapy component. If another treatment is proposed, ask whether it happens before, during or after this course and who will coordinate the appointments.

Testing continues after panitumumab

Some changes can outlast the infusion course.

Before and during treatment

Confirm the DPD result before fluorouracil unless already tested. Blood count, kidney and liver tests are required before each cycle. Skin, eyes and electrolytes including magnesium and calcium need monitoring during panitumumab treatment.

For two months after the last dose

Arrange monthly electrolytes, magnesium and calcium for two months after the final panitumumab dose. Report cramps, weakness, palpitations or other new symptoms; do not wait for a routine test if unwell.

Conditional heart and anticoagulant checks

A baseline ECG is needed if cardiac function is compromised; later heart investigations depend on symptoms. If taking warfarin, INR is checked as clinically indicated. Confirm who arranges and reviews your INR tests.

Pauses, recovery and follow-up

Your team may pause or change one or more medicines. Do not restart or change doses yourself.

Decisions before restarting

The team checks the severity and recovery of side effects, previous episodes and the doses you actually received. A standard table dose does not automatically override a prior individual reduction.

Keep testing appointments

Blood tests may continue during a treatment pause. Keep monthly electrolyte, magnesium and calcium checks for two months after the last panitumumab dose. Scans and tumour-marker timing follow the consultant plan; these documents do not impose a universal scan interval.



Check every medicine and supplement

Show the team all prescribed, over-the-counter and herbal medicines before treatment and before starting anything new. Certain antiviral medicines, including brivudine and sorivudine, must not be combined with fluorouracil. Tell the team if you take warfarin or phenytoin; extra monitoring may be needed. Do not stop or change medicines yourself.



Irinotecan medicine interactions

Ask the oncology pharmacist before taking grapefruit, herbal products or any new medicine. Some medicines change irinotecan exposure. Prochlorperazine should not be taken on the same day as irinotecan; confirm your alternative prescribed nausea plan with the team. Tell the team about known Gilbert syndrome before the first treatment.

Questions worth asking

Why do I need a RAS test?

RAS is part of the growth-signalling pathway inside cancer cells. If it is changed by a mutation, blocking EGFR is unlikely to help. Your team needs a validated wild-type RAS result before panitumumab. Other tumour tests and the treatment aim also affect the choice.

Is this the same as checkpoint immunotherapy?

Panitumumab is a targeted antibody against EGFR. It works differently from checkpoint treatments used in some other cancer regimens. Ask your team which treatment class and side effects apply to you.

Does a rash mean the treatment is working?

A rash is a recognised drug effect, but it cannot tell you whether your cancer is responding. Report it early so it can be treated. Scans, symptoms and clinical review guide the response assessment.

Will I need radiotherapy as part of this cycle?

These panitumumab regimens do not contain a scheduled radiotherapy course. If radiotherapy is recommended separately, your oncology teams will explain its purpose and timing. Do not assume it occurs on infusion or pump days.

Does having more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be treated. Your team assesses benefit using the tests and review plan appropriate to your cancer.

Why might a dose be delayed or reduced?

Your team may allow recovery, investigate a symptom or reduce the chance of lasting harm. Ask which drug changes, why, and when review is planned. A delay does not itself mean treatment has failed.

What should I say when calling?

Give the regimen name, last treatment date, symptoms and when they started. Explain whether you can eat, drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect details.

> **Try explaining the plan back:** Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Panitumumab + FOLFIRI

Active medicines: panitumumab + irinotecan + folinic acid + fluorouracil

Emetogenic risk: moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Day 1 before chemotherapy Dose may be adjusted for individual/QT risk.
Dexamethasone	8 mg orally daily	Days 1–3 Reconcile all steroid prescriptions.
Metoclopramide	10 mg orally up to three times daily as prescribed	As required on subsequent days Confirm suitability, duration and contraindications.
Atropine	250 micrograms subcutaneously	Before irinotecan per NCCP prophylaxis Check contraindications including glaucoma; this does not treat delayed diarrhoea. Use caution in asthma, as specified in the NCCP acute cholinergic-syndrome advice.

Additional phase-specific support

- No routine panitumumab premedication unless previous hypersensitivity/local policy indicates.
- Written delayed-diarrhoea plan and urgent fever/dehydration advice; do not confuse with acute cholinergic management.
- Skin care and electrolytes; monthly Mg/Ca/electrolytes for two months after last panitumumab.
- Supply a written irinotecan delayed-diarrhoea rescue prescription: NCCP reference loperamide 4 mg initially, then 2 mg every 2 hours; continue for 12 hours after the last liquid stool, never longer than 48 hours at this high-dose schedule. Start advised electrolyte-containing fluids at the first liquid stool and contact the team early. Fever, dehydration, severe pain or persistent symptoms require urgent oncology review.
- Irinotecan treatment-related diarrhoea must fully resolve before restart; a table-grade threshold alone does not clear the next dose.
- If treatment is delayed for cytopenia, follow the source-specific weekly blood-count checks and maximum delay; assess the antibody and chemotherapy components separately when indicated.
- Medication review: NCCP contraindicates concomitant brivudine, sorivudine and analogues with fluorouracil; review dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-affecting medicines, phenytoin toxicity/levels and warfarin international normalised ratio (blood-clotting test) (INR).
- Review CYP3A inducers/inhibitors and grapefruit; avoid prochlorperazine on the same day as irinotecan. Gilbert syndrome warrants consideration of a reduced starting irinotecan dose; no universal percentage is supplied.
- EMA Vectibix also requires interruption for intolerable dermatologic reactions; a grade below 3 does not automatically clear treatment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eyes

Panitumumab

New eye pain or visual change needs prompt advice.

2 Mouth

Fluorouracil + folinic acid

Report ulcers or pain affecting eating and drinking.

3 Chest

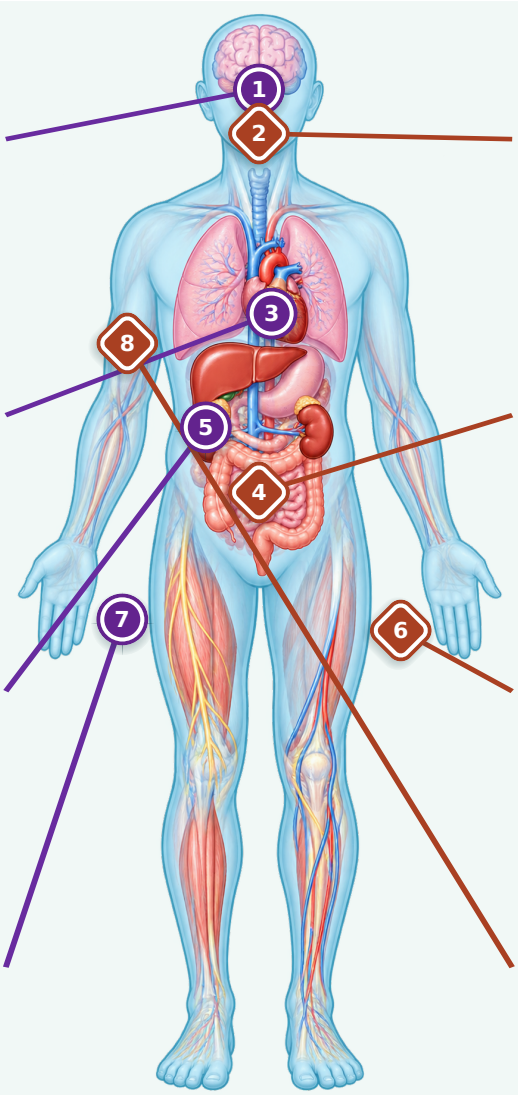
Fluorouracil

New chest pain or breathlessness needs urgent assessment.

4 Bowel

Irinotecan / fluorouracil

Report diarrhoea, increased stoma output or severe abdominal pain.



5 Hydration / minerals

Panitumumab + chemotherapy

Reduced urine, cramps or weakness need advice; keep your blood-test appointments.

6 Skin / nails

Panitumumab

Report rash, cracks or painful nails early.

7 Hands / feet

Fluorouracil

Report painful redness, peeling or difficulty using your hands or walking.

8 Pump / line

Treatment delivery

Pain, leakage, swelling or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathing difficulty, eye pain or blurred vision, blistering or infected skin, severe diarrhoea or dehydration promptly. Also report new chest pain, fever or a line/pump problem immediately.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00448 v5 · official regimen**
   https://healthservice.hse.ie/documents/6480/448_Panitumumab_plus_FOLFIRI.pdf
2. **Panitumumab product information ↗**
   https://www.medicines.org.uk/emc/product/6178/smpc
3. **eviQ: treatment through an implanted port**
   https://www.eviq.org.au/p/507
4. **eviQ: treatment through a peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
5. **eviQ: elastomeric infusion pump care**
   https://www.eviq.org.au/p/502
6. **Panitumumab patient medicine leaflet**
   https://www.medicines.org.uk/emc/product/6178/pil
7. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **20050181 RAS analysis — primary research report**
   https://pubmed.ncbi.nlm.nih.gov/26341920/
9. **eviQ: patient explanation (NCCP defines your regimen schedule)**
   https://www.eviq.org.au/medical-oncology/upper-gastrointestinal/gastric-and-oesophageal-metastatic/4043-advanced-or-metastatic-folfiri-modified-fl/patient-information
10. **NCI: nausea and vomiting**
    https://www.cancer.gov/about-cancer/treatment/side-effects/nausea-vomiting
11. **NCCP emetogenic classification V6**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
12. **NCCP NCIS Medical Oncology antiemetics V8, Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
13. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
    https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq